

Procedure Assessment for Resident – Hemorrhoidectomy

Resident Resident year ...4.....Operation ...Hemorrhoidectomy.....

Date _____ Resident _____ Staff _____

	F – Fails M – Meets expectation E - Exceeds	F – Fails M – Meets expectation E - Exceeds	Staff comments
Preparation : Short brief an information : Hx, PE, Lab, X-ray	F M E	F M E	
Time out 1. Antibiotics prophylaxis 2. What are the critical or non-routine steps? 3. How long will the case take? 4. What is the anticipated blood loss?	F M E	F M E	
Operation : Position setting : Lithotomy/ Prone Jack Knife	F M E	F M E	
Incision and approach	F M E	F M E	
Relevant anatomy : Identify 1. Internal sphincter 2. External sphincter 3. Plane of Dissection 4. Hemorrhoidal Pedicle Ligation	F M E	F M E	
Anticipates steps in procedure 1. Positioning setting 2. Proctoscopy and PR examination 3. Hemorrhoidectomy technique 4. Bleeding control	F M E	F M E	
Knowledge of critical points of operation 1. Planned hemorrhoidectomy 2. Identify internal sphincter 3. Bleeding control 4. Normal mucosal gap to prevent stricture	F M E	F M E	
Demonstrates proper handling of tissue and instrument	F M E	F M E	
Good basic skills : suture/knot tying	F M E	F M E	
Follows a logical sequence in case	F M E	F M E	
Bleeding control and wound dressing	F M E	F M E	
Follow-Up : Completed Sign-out : Nurse verbally confirms (procedure, instrument, specimen, any problems)	F M E	F M E	
Complete post-operative orders	F M E	F M E	
Saw patient in recovery room	F M E	F M E	

Resident self assessment : What did you learn from this case : _____

Staff assessment : What would you recommend to help the resident improve their skills/ preparation : _____

Overall assessment : F M E

Signature (Staff)